MEDRECORDS

Stress-Test Memo

Hard questions, honest answers

Purpose

This memo collects the hardest questions an experienced investor, a clinical advisor, and a regulator are likely to ask about MedRecords, and gives the strongest honest answer we can give today. The goal is not to spin. The goal is to know our weak points before someone else finds them.

Each question is answered in three parts: the steel-manned objection, our actual answer, and what we still need to prove.

1. Investor Pushback

1.1 Hasn't this been tried already? Why hasn't anyone built it?

The objection: EMR has been attempted in Nigeria for two decades, by HMS vendors, by government programs, by foreign donors. None has produced a nationwide unified record. If it were possible, someone with more capital and political access would already have done it.

Our answer: What has been tried is institutional EMR — software that lives inside one hospital, owned by that hospital. That is a fundamentally different product from a patient-owned, cross-institutional record. The cross-institutional product was not feasible until three things became true recently: NIN coverage crossed a usable threshold, smartphone penetration in target segments became sufficient, and NDPA created a regulatory framework that legitimizes the data layer. The window is now open, and the previous failures inform our approach (patient-owned not institution-owned, standards-first, pharmacy-led adoption).

What we still need to prove: That the window is actually open. The clearest evidence will be early adoption in the design-partner phase: are we able to onboard pharmacies and patients at the rates our model assumes.

1.2 Who actually pays you in the first 24 months?

The objection: Patients won't pay. Hospitals will resist. Government moves at government speed. Where does the early revenue come from?

Our answer: Pharmacies pay first, at a modest per-branch SaaS fee, because they get verification, controlled-substance compliance, and customer loyalty in return. GP clinics pay second once pharmacy data exists in their geography. The first material revenue line is a payer (HMO) contract in late phase 2 / early phase 3. We are not relying on government revenue in the first 24 months.

What we still need to prove: That pharmacies will pay the price points we assume. We will know this from beta pricing experiments in months 3–9. If pharmacies will only adopt free, our seed runway must extend to reach the payer revenue line, which adds 6–9 months.

1.3 Why won't a hospital group or HMS vendor build this themselves?

The objection: Helium Health, Reliance, or a vertically integrated HMO could decide to extend their existing footprint into the record layer and crush a standalone startup.

Our answer: Institutional players are structurally disadvantaged at building a patient-owned record. Their existing customers (hospitals) will not give up record ownership voluntarily, and a patient-owned record requires brand trust that is positioned against institutional power rather than aligned with it. The closest competitive threat is a hospital group spinning out a patient app, but those efforts struggle because patients see the brand as the hospital's, not their own. We are betting on neutrality and patient ownership being the durable advantage.

What we still need to prove: That neutrality and patient ownership are recognized as a durable advantage by patients and providers. Measured by patient claim rates and provider-side perception in adoption interviews.

1.4 Aren't you just one regulatory change away from being shut down or commoditized?

The objection: The Federal Ministry of Health could mandate a state-owned national EHR tomorrow and you become obsolete.

Our answer: Two answers. First, the FMOH has been talking about a national EHR for many years; the gap between aspiration and execution is large, and we do not believe a turnkey government solution is imminent. Second, even if one is built, the historical pattern in similar markets (India's ABDM, Estonia's eHealth) is that the government builds standards and APIs and lets private providers deliver the patient and provider experience. We are positioning to be that private layer: we participate in standards, we comply with whatever the government produces, and we make our product better than what the public sector can deliver on its own. The risk is real but the timeline is favorable.

What we still need to prove: Active engagement with FMOH digital health unit beginning phase 0. We need to be in the room when standards are decided.

1.5 Healthcare is a slow, low-margin, distribution-heavy business. Why is this a venture-scale opportunity?

The objection: Even if you succeed, are the unit economics big enough for a venture return?

Our answer: The patient app and provider tools alone are not venture-scale. The payer revenue line is. Per-member-per-month data access to an HMO with hundreds of thousands of lives is a recurring contract worth hundreds of thousands of USD annually each, and there are dozens of such institutions in Nigeria. Add ECOWAS expansion and the addressable PMPM revenue is well into the hundreds of millions. The other lines — pharmacies, clinics, exports — fund the build to get there.

What we still need to prove: First HMO contract economics. Until we have a signed payer at the prices our model assumes, the venture-scale claim is a hypothesis.

1.6 Why this team?

The objection: Healthcare startups in emerging markets fail when the team lacks either deep clinical credibility or deep distribution muscle, and most fail on both.

Our answer: We need both, and that shapes hiring. The clinical credibility comes from a head of clinical operations who is a registered MDCN doctor, a clinical advisory board with named senior figures, and design partnerships with respected institutions. The distribution muscle comes from a head of field operations with experience selling SaaS to pharmacies (Lipa Later, Yellow Card, or similar fintech distribution backgrounds translate well). The technical team is recruited against scarce profiles: senior engineers who have shipped offline-first mobile in low-connectivity markets.

What we still need to prove: Until the team is fully hired, this is the largest soft risk. Investor confidence in the team is part of the seed thesis.

1.7 What's the killer scenario where this all falls apart?

The objection: Tell me the way this fails.

Our answer: The most likely failure mode is this: we build a beautiful product, pharmacies adopt enthusiastically, but the resulting data doesn't change clinician behavior — GPs continue writing on paper and ignore the platform, and the cross-provider continuity rate never gets above single digits. We are then a pharmacy verification SaaS, which is a real business but not the company we set out to build. The mitigation is to

design clinician adoption with the same rigor as pharmacy adoption, and to use the first 12 months of clinical advisory and design-partner feedback to find the workflow integration that genuinely saves time.

What we still need to prove: Provider time-to-first-record and ongoing usage rates are the early-warning metrics for this scenario.

2. Regulator Pushback

2.1 NDPC: How will you handle the right to erasure given medical record retention obligations?

The objection: Patients have a right to erasure under NDPA. Doctors have a duty to retain records under the National Health Act and MDCN ethics. These are in tension. What gives?

Our answer: We treat the right to erasure as a right to restrict processing, not a right to destroy records. A patient may request that their record be no longer accessible to providers or used in our systems. Where that conflicts with mandatory retention (typically 7 years from last encounter, longer for minors), the record is moved to restricted storage, inaccessible to clinicians, accessible only to the patient and to legal process. Erasure of restricted records occurs only after the legal retention period expires. This balance is documented in our privacy policy and was reviewed with NDPA counsel.

What we still need to prove: Formal review of this approach with NDPC during data controller registration. We expect this to be acceptable but want it on the record.

2.2 NDPC: How can patient consent be "freely given" if denying consent means not getting care?

The objection: If a patient cannot receive treatment without granting MedRecords consent, the consent is not truly free.

Our answer: Care is never conditional on MedRecords consent. A provider on the platform can decline to use the platform for a specific patient who chooses not to grant consent, and the patient receives care under the provider's pre-existing paper or HMS workflow. MedRecords is an option, not a requirement. We will document this explicitly in provider terms of service and audit for compliance.

What we still need to prove: Audit mechanism — how do we confirm in practice that providers are not refusing care to non-consenting patients? Likely a combination of patient feedback channels and randomized facility audits.

2.3 MDCN/PCN: What is your liability stance if data on MedRecords is incorrect and a clinical decision is harmed?

The objection: Doctors are liable for clinical decisions. If your platform shows the wrong allergy and a patient is harmed, who is responsible?

Our answer: Clinical decisions are the responsibility of the clinician. Our terms of service make explicit that MedRecords is a record-keeping platform, not a clinical decision-support system, and that providers must verify critical information with the patient. Authorship is preserved: the record shows which provider entered which data and when, with audit. If incorrect data on the platform contributes to harm, liability flows to whoever authored the incorrect entry, not to MedRecords. This is the same legal structure as a hospital's paper chart.

What we still need to prove: Test cases with MDCN ethics committee and the Medical Defence Union or local indemnity provider equivalents. We need their concurrence in writing.

2.4 FMOH: How does MedRecords align with the national digital health strategy?

The objection: We don't want a private platform to fragment the national health data architecture. How does your platform serve national public health goals?

Our answer: We commit to: (a) publishing our Nigeria FHIR Implementation Guide as an open standard contributed to the digital health community, (b) providing public health surveillance feeds (de-identified, aggregated) to FMOH at no charge for notifiable diseases and immunization, (c) interoperability with NHIA's enrolment and claims systems, and (d) participation in the national digital health technical working group. We are not building a walled garden. We are building a private operator that complies with and contributes to the public architecture.

What we still need to prove: Formal MOU with FMOH digital health unit. Targeted for late phase 1 / early phase 2.

2.5 NDPC: What happens to the data if MedRecords is acquired or shuts down?

The objection: A platform holding 25 million people's records cannot just disappear. What is the continuity plan?

Our answer: We will publish a data continuity policy with three commitments. First, patients always have the right to export their full record in standard FHIR format and as signed PDFs. Second, in the event of acquisition, the data controller relationship continues under the new entity, with patient notification and a window to export and withdraw if the new entity is unacceptable. Third, in the event of shutdown, we commit to a transition period of at least 12 months during which patients can export their records, and to handing custody of the underlying records (with patient consent) to an FMOH-designated or NDPC-designated successor. This is documented in the platform's articles and in our terms of service.

What we still need to prove: Formalization of the shutdown plan with NDPC. Many SaaS continuity policies are vague; ours should not be.

3. Clinical Advisor Pushback

3.1 Will any actual doctor want to use this?

The objection: Most clinical software is built by people who have never sat in a Nigerian outpatient clinic. Why is yours different?

Our answer: Three commitments. First, the clinical operations function is led by a registered MDCN doctor from day one, not as an advisor but as an operator. Second, every UI flow used by clinicians is tested with a clinical advisory panel of practicing Nigerian doctors before it ships. Third, we measure time-to-first-record and encounters-per-clinician-per-day from beta onward and treat workflow speed as a launch-blocking criterion. If a clinician cannot finish an encounter in MedRecords faster than they finish it on paper, we delay rather than ship.

What we still need to prove: Time-to-first-record and encounters per clinician are the live metrics that will confirm or refute this.

3.2 What about the doctor who deliberately does not want their notes seen by other doctors?

The objection: Doctors compete. Some don't want a referred patient's record to be visible to the receiving specialist because it reveals their practice patterns. They will sabotage the platform.

Our answer: We acknowledge this is real. Mitigations: (a) the patient owns the record and is the one granting access, so a doctor cannot unilaterally restrict visibility; (b) we are not requiring detailed clinical notes — structured fields (diagnosis, prescription) are mandatory but the SOAP-style note is the provider's intellectual product and they may choose how to share it; (c) the patient sees who has accessed their record, creating a

transparency norm that pressures all providers; (d) in adversarial cases, the patient is the arbiter, and they can revoke access from any provider. We do not solve professional rivalry; we make it less expensive to the patient.

What we still need to prove: Adoption among specialists who receive referrals is the test. If specialists refuse to engage because GPs withhold useful context, we adjust the structured-field requirements.

4. Patient Advocate Pushback

4.1 Are you creating a surveillance system for marginalized populations?

The objection: Aggregated health data on poor or marginalized Nigerians could be used for purposes those populations would not consent to — insurance discrimination, employment screening, government surveillance.

Our answer: We treat this seriously. Four protections. First, the platform never sells identified data — full stop, in our articles. Second, aggregated de-identified data is shared only with FMOH for public health and with research entities under ethics-committee approval; patients can opt out. Third, no government agency receives identified data without patient consent or lawful court order. Fourth, special-category data (HIV, mental health, reproductive) gets the field-level encryption and additional consent layer described in the consent model. We will publish an annual transparency report disclosing aggregate access patterns and any government requests.

What we still need to prove: The annual transparency report and the independent audit of de-identification methodology are the proof points. To be designed before launch and reviewed with civil-society advocates.

4.2 Patients in rural areas with no smartphone — are they second-class users?

The objection: Your concept skews mobile-first. Most Nigerians who most need this don't have a smartphone.

Our answer: USSD is in v1 scope, not v2. The fallback is genuinely usable for the core flows (consent grant, prescription verification, record summary). The patient experience is degraded, but the medical record is not — providers can still write the same structured data into their record. Over time, smartphone penetration improves; in the meantime, USSD plus provider-led data entry covers the population that the app cannot.

What we still need to prove: USSD usability and conversion rates from feature-phone users in the beta. If USSD adoption is below 20% of feature-phone patients invited, we have a real reach problem and need a better fallback (community health worker tablets, partnership with HMOs that subsidize devices).

5. What This Memo Does Not Cover

Three categories of pushback are deliberately not addressed in this memo because they need more work first:

- Specific financial projections. The unit-economics framework is in the business model document; full bottoms-up financial model is a separate workstream.
- Detailed competitive deep-dive on each Nigerian and African digital health player. This requires primary research with current customers of those players and is scoped for the seed-funded period.
- Technical architecture details on scale beyond v1. We have a viable architecture for the v1 numbers; the architecture for 10M+ patient accounts will require revisiting some choices, and that is a Phase 4 exercise.

6. Update Cadence

This memo is updated quarterly during the seed and Series A period. New questions surfaced in investor meetings, regulator conversations, or clinical advisory sessions are added. Answers that have been validated by evidence are marked as such; answers that remain assumptions are flagged for testing.